

Patient Name : Mr.SWAPAN BANERJEE
Age/Gender : 56 Y 11 M 6 D/M
UHID/MR No : FSBP.0000020230
Visit ID : FSBROPV31858
Ref Doctor : Self
Emp/Auth/TPA ID : 123

Collected : 28/Oct/2025 10:58AM
Received : 28/Oct/2025 11:28AM
Reported : 28/Oct/2025 05:20PM
Status : Final Report
Sponsor Name : SGD 369 HCL

DEPARTMENT OF HAEMATOLOGY

AHC ABOVE 40Y MALE - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
BLOOD GROUP ABO AND RH FACTOR , WHOLE BLOOD EDTA				
BLOOD GROUP TYPE	O			Forward Grouping with Slide
Rh TYPE	POSITIVE			Forward Grouping with Slide

Comment:

1. This tests determines ABO & Rh blood groups (testing for other blood group systems not performed) through immunological reaction between RBC antigen & antibody.
2. ABO system also has Subgroups of A, B and rare phenotype as Bombay blood group which requires further testing and required recommendations as per the case will be provided.
3. Rh system in certain individual can have weak or partial Rh D expression which can result in weaker agglutination reactions and hence all Rh D Negative groups need to be further cross verified using Rh Du testing.
4. In case of Newborn - Only forward typing is performed, reverse typing is not performed, since the antibodies are not fully formed. Hence it is recommended to re-test blood grouping after 6 months.
5. In certain cases History of Recent blood transfusion (within 3-4mths), of bone marrow transplantation, certain drugs (especially monoclonal antibody) & certain malignancies may interfere with interpretation of blood grouping.
6. It is always recommended for reconfirmation of the Blood Group along with cross matching before blood transfusion.

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HEMOGRAM.- , WHOLE BLOOD EDTA				
HAEMOGLOBIN	13.5	g/dL	12-16	Colorimetric
RBC COUNT	4.18	10 ¹² /L	3.5-5.5	IMPEDANCE
PCV	39.5	%	36-48	ANALYSER CALCULATION
MCV	96.7	fL	80-99	ANALYSER CALCULATION
MCH	32.2	pg	26-32	ANALYSER CALCULATION
MCHC	34.1	g/dL	30-36	ANALYSER CALCULATION
R.D.W- CV	12.2	%	11.5-14.5	ANALYSER CALCULATION
R.D.W-SD	38.0	fL	37-54	ANALYSER CALCULATION
TOTAL LEUCOCYTE COUNT (TLC)	8.3	10 ⁹ /L	4-10	IMPEDANCE
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHIL	70	%	40-80	Microscopy
LYMPHOCYTE	21	%	20-40	Microscopy
MONOCYTE	08	%	2-10	Microscopy
EOSINOPHIL	01	%	1-6	Microscopy
BASOPHIL	00	%	0-1	Microscopy
Neutrophil lymphocyte ratio (NLR)	3.33			Calculated
PLATELET COUNT	245	10 ⁹ /L	100-300	Electrical impedence/MICROSCOPY
MPV	10.1	fL	7.4-10.4	Calculated
PDW	13.8	—	10.0 - 17.0	ANALYSER CALCULATION

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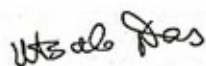
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ERYTHROCYTE SEDIMENTATION RATE (ESR)	15	mm/h(1st Hr.)	0-14	Method (Westergren & Manual)
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PERIPHERAL SMEAR

RBC NORMOCYTIC NORMOCHROMIC
WBC WITHIN NORMAL LIMITS
PLATELETS ARE ADEQUATE ON SMEAR



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DEPARTMENT OF BIOCHEMISTRY

AHC ABOVE 40Y MALE - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, FASTING , NAF PLASMA	120	mg/dL	70-110	GOD- Trinder

Comment:

As per American Diabetes Guidelines, 2023

Fasting Glucose Values in mg/dL	Interpretation
70-100 mg/dL	Normal
100-125 mg/dL	Prediabetes
≥126 mg/dL	Diabetes
<70 mg/dL	Hypoglycemia

Note:

- 1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL and/or a random / 2 hr post glucose value of > or = 200 mg/dL on at least 2 occasions.
2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, POST PRANDIAL (PP), 2 HOURS , SODIUM FLUORIDE PLASMA (2 HR)	128	mg/dL	90-140	GOD- Trinder

Comment:

It is recommended that FBS and PPBS should be interpreted with respect to their Biological reference ranges and not with each other.

Conditions which may lead to lower postprandial glucose levels as compared to fasting glucose levels may be due to reactive hypoglycemia, dietary meal content, duration or timing of sampling after food digestion and absorption, medications such as insulin preparations, sulfonylureas, amylin analogues, or conditions such as overproduction of insulin.

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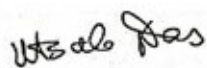


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AHC ABOVE 40Y MALE - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C , WHOLE BLOOD EDTA				
HbA1c	5.5	%	4.5-6.5	HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	111	mg/dL		Calculated

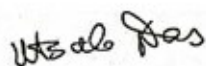
Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

REFERENCE GROUP	HBA1C %
NON DIABETIC	<5.7
PREDIABETES	5.7 – 6.4
DIABETES	≥ 6.5
DIABETICS	
EXCELLENT CONTROL	6 – 7
FAIR TO GOOD CONTROL	7 – 8
UNSATISFACTORY CONTROL	8 – 10
POOR CONTROL	>10

Note: Dietary preparation or fasting is not required.

- HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
- Trends in HbA1C values is a better indicator of Glycemic control than a single test.
- Low HbA1C in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
- Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
- In cases of Interference of Hemoglobin variants in HbA1C, alternative methods (Fructosamine) estimation is recommended for Glycemic Control
 - HbF >25%
 - Homozygous Hemoglobinopathy.
(Hb Electrophoresis is recommended method for detection of Hemoglobinopathy)



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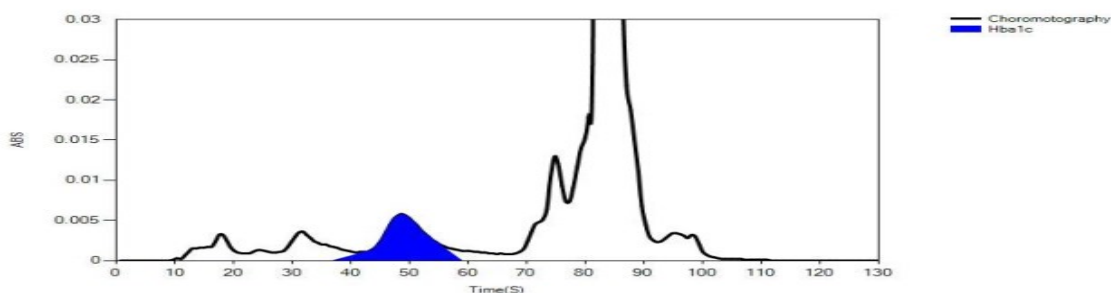
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AHC ABOVE 40Y MALE - SARAT BOSE ROAD

LIFOTRONIC Graph Report

Name :	Case :	Patient Type :	Test Date : 28/10/2025 14:58:56
Age :	Department :	Sample Type : Whole Blood EDTA	Sample Id : SBR251001566
Gender :			Total Area : 12576

Peak Name	Retention Time(s)	Absorbance	Area	Result (Area %)
HbA0	83	3824	11442	91.0
HbA1c	48	58	633	5.5
La1c	31	36	274	2.4
HbF	24	13	53	0.5
Hba1b	17	34	109	0.9
Hba1a	15	17	65	0.6



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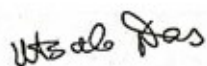
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Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) WITH GGT , SERUM				
BILIRUBIN, TOTAL	0.74	mg/dL	ADULT 0.1 1.20~INFANT 1.2 12	(DIAZO)
BILIRUBIN CONJUGATED (DIRECT)	0.22	mg/dL	0.0-0.3	(DIAZO)
BILIRUBIN (INDIRECT)	0.52	mg/dL	ADULT 0 1.1~INFANT 0.6 10.5	Calculated
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	35.12	U/L	0-45	(IFCC)
ALANINE AMINOTRANSFERASE (ALT/SGPT)	38.48	U/L	0-45	(IFCC)
AST (SGOT) / ALT (SGPT) RATIO	0.91	RATIO		Calculated
ALKALINE PHOSPHATASE	66.04	U/L	41 - 137	IFCC
PROTEIN, TOTAL	6.1	g/dL	6-8.3	Biuret
ALBUMIN	4.11	g/dL	3.5 - 5.5	Bromocresol green
GLOBULIN	1.99	g/dL	2.8-3.2	Calculated
A/G RATIO	2.07	RATIO	1.2-2.1	Calculated
GAMMA GLUTAMYL TRANSPETIDASE (GGT)	55.42	U/L	0-50	GLUPA - C

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

1. Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries.*ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1 In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilsons's diseases, Cirrhosis, but the increase is usually not >2. Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.

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2. Cholestatic Pattern:*ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex.*Bilirubin elevated- predominantly direct , To establish the hepatic origin correlation with elevated GGT helps.
3. Synthetic function impairment:*Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
4. Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

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Test Name	Result	Unit	Bio. Ref. Interval	Method
UREA. , SERUM	21.08	mg/dL	13-45	Urease
BLOOD UREA NITROGEN , SERUM	9.85	mg/dL	6-21	Calculated

Test Name	Result	Unit	Bio. Ref. Interval	Method
CREATININE - SERUM , SERUM				
CREATININE	1.02	mg/dL	0.6 - 1.4	Jaffe

Test Name	Result	Unit	Bio. Ref. Interval	Method
URIC ACID , SERUM	5.92	mg/dL	3.6-7.2	Uricase

Comments:-

Uric acid is an end product of purine catabolism. Most uric acid is synthesised in the liver & from the intestine. Two thirds of uric acid is excreted by the kidneys.

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Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , SERUM				
TOTAL CHOLESTEROL	173.49	mg/dL	Desirable <200~Borderline 200- 239~High Risk:>240	Trinders
TRIGLYCERIDES.	157.92	mg/dL	35-170	GPO-Trinders
HDL CHOLESTEROL.	27.62	mg/dL	Low Risk>50~Moderate FSR 35-50~High Risk<30	
LDL CHOLESTEROL..	114.3	mg/dL	Optimal<100~Boderline High:130- 160~High:161- 190~Very High:>190	Calculated
NON - HDL CHOLESTEROL	145.87	mg/dl	<130	Calculated
VLDL CHOLESTEROL	31.6	mg/dL	<40	Calculated
CHOL / HDL RATIO	6.28		0-5	Calculated
LDL / HDL RATIO	4.14		0-3.55	Calculated

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DEPARTMENT OF IMMUNOLOGY

AHC ABOVE 40Y MALE - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
THYROID STIMULATING HORMONE (TSH) , SERUM	3.8	µIU/mL	0.25-5	ELISA

Comment:

TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH is a labile hormone & is secreted in a pulsatile manner throughout the day and is subject to several non-thyroidal pituitary influences. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, caloric intake, medication & circulating antibodies.

It is important to confirm any TSH abnormality in a fresh specimen drawn after ~ 3 weeks before assigning a diagnosis, as the cause of an isolated TSH abnormality.

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN D (25 - OH VITAMIN D) , SERUM	20.1	ng/mL	30-100	ELISA

Comment:

BIOLOGICAL REFERENCE RANGES

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL)
DEFICIENCY	<10
INSUFFICIENCY	10 – 30
SUFFICIENCY	30 – 100
TOXICITY	>100

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The biological function of Vitamin D is to maintain normal levels of calcium and phosphorus absorption. 25-Hydroxy vitamin D is the storage form of vitamin D. Vitamin D assists in maintaining bone health by facilitating calcium absorption. Vitamin D deficiency can also cause osteomalacia, which frequently affects elderly patients.

Vitamin D Total levels are composed of two components namely 25-Hydroxy Vitamin D2 and 25-Hydroxy Vitamin D3 both of which are converted into active forms. Vitamin D2 level corresponds with the exogenous dietary intake of Vitamin D rich foods as well as supplements. Vitamin D3 level corresponds with endogenous production as well as exogenous diet and supplements. Vitamin D from sunshine on the skin or from dietary intake is converted predominantly by the liver into 25-hydroxy vitamin D, which has a long half-life and is stored in the adipose tissue. The metabolically active form of vitamin D, 1,25-di-hydroxy vitamin D, which has a short life, is then synthesized in the kidney as needed from circulating 25-hydroxy vitamin D. The reference interval of greater than 30 ng/mL is a target value established by the Endocrine Society.

Decreased Levels:- Inadequate exposure to sunlight, Dietary deficiency, Vitamin D malabsorption, Severe Hepatocellular disease., Drugs like Anticonvulsants, Nephrotic syndrome.

Increased levels:- Vitamin D intoxication.

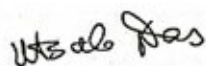
Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN B12 , SERUM	509	pg/mL	200-835	ELISA

Comment:

Population based data reflecting exact scenario of vitamin B12 levels in Indian population is still evolving, however, different studies reporting a deficiency in adults, pregnant women and children ranging from 16% to 77% with average of about 47%. This high incidence is attributed to vegetarian food habits of large majority of Indian population.

Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency. B12 levels in the range of 150 to 190 pg/ml may not be associated with any clinical manifestations, while B12 levels below 100 pg/ml are often associated with clinical symptoms. However, for an individual based on other co-morbid conditions or other nutritional deficiency (especially folate) the manifestations can vary accordingly.

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If clinical symptoms suggest deficiency, measurement of active vitamin B12, MMA and homocysteine should be considered as further workup.

Test Name	Result	Unit	Bio. Ref. Interval	Method
TOTAL PROSTATIC SPECIFIC ANTIGEN (tPSA) , SERUM	0.29	ng/mL	0-5.5	ELISA



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DEPARTMENT OF CLINICAL PATHOLOGY
AHC ABOVE 40Y MALE - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE), , URINE				
PHYSICAL EXAMINATION				
VOLUME	30	ml		Volumetric
COLOUR	PALE STRAW		Clear To Pale Yellow	Macroscopy
APPEARANCE	SLIGHT HAZY			Macroscopy
CLINICAL EXAMINATION				
SP. GRAVITY	1.015		1.005-1.03	Dipstick
pH	5.0			Dipstick
URINE PROTEIN	NEGATIVE		NEGATIVE	Dipstick
GLUCOSE	NEGATIVE		Negative	Dipstick
URINE BILIRUBIN	NEGATIVE		NEGATIVE	Dipstick
URINE KETONES	NEGATIVE		NEGATIVE	Dipstick
UROBILINOGEN	Normal		NORMAL	Dipstick
OCCULT BLOOD	NEGATIVE		NEGATIVE	DIPSTICK
NITRITE	NEGATIVE		NEGATIVE	Dipstick
LEUCOCYTE ESTERASE	NEGATIVE		NEGATIVE	Dipstick
MICROSCOPIC EXAMINATION				
PUS CELLS	0-1	/hpf	0-5	Microscopy
RBC	ABSENT	/hpf	0-2	Microscopy
EPITHELIAL CELLS	0-1	/hpf	<10	Microscopy
CASTS	NIL			Microscopy
CRYSTALS	NOT FOUND		ABSENT	Microscopy
BACTERIA	NOT FOUND		Absent	Microscopy

***** End Of Report *****

Result/s to Follow:

Page 16 of 17

Utsab Das

Dr Utsab Das
MBBS, MD(PATH)
CONSULTANT

SIN No:SBR251001570



Patient Name : Mr.SWAPAN BANERJEE
Age/Gender : 56 Y 11 M 6 D/M
UHID/MR No : FSBR.0000020230
Visit ID : FSBROPV31858
Ref Doctor : Self
Emp/Auth/TPA ID : 123

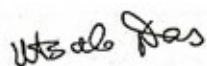
Collected : 28/Oct/2025 10:58AM
Received : 28/Oct/2025 11:28AM
Reported : 28/Oct/2025 05:19PM
Status : Final Report
Sponsor Name : SGD 369 HCL

DEPARTMENT OF CLINICAL PATHOLOGY

AHC ABOVE 40Y MALE - SARAT BOSE ROAD

STOOL ROUTINE EXAMINATION

Page 17 of 17



Dr Utsab Das
MBBS, MD(PATH)
CONSULTANT

SIN No:SBR251001570



Patient ID: 012510280293
Name: Mr. SWAPAN BANERJEE
Age/Gender: 56 Y/Male
Referred By: Self

Primary Sample Collection Time: 28/Oct/2025 14:44:34
Collection Date/Time: 28/Oct/2025 03:15PM
Received Date/Time: 28/Oct/2025 03:24PM
Approved Date/Time: 28/Oct/2025 06:13PM

DEPARTMENT OF CLINICAL PATHOLOGY

Test Description	Observed Value	Unit	Method	Biological Ref. Interval
STOOL FOR ROUTINE EXAMINATION:				
PHYSICAL EXAMINATION:				
COLOUR	BROWNISH			Whitish
CONSISTENCY	SEMI FORMED			
Mucus	present			Absent
VISIBLE BLOOD	absent			
CHEMICAL EXAMINATION:-				
REACTION / pH	ACIDIC			5.80-7.50
STOOL REDUCING SUBSTANCE	absent		Chemical	Negative
OCCULT BLOOD	NEGATIVE			
MICROSCOPIC EXAMINATION:-				
PUS CELLS	2-3	/hpf		NIL
RBC	NOT FOUND	/HPF		
MACROPHAGE	NOT FOUND	%		
OVA	NOT FOUND	MICROSCOPIC		NIL
CYST	NOT FOUND	MICROSCOPIC		NIL
TROPHOZOITES	NOT FOUND			
Larvae	NOT FOUND			
PARASITES:	NOT FOUND	MICROSCOPIC		Not Seen
VEG. CELLS	PRESENT			

Disclaimer: The test result mentioned here should be interpreted in view of clinical condition of the patient. In case of any clinical suspicion regarding any parameter, repeat test with fresh sample essential to conclude.

*** End Of Report ***

APOLLO CLINIC
44/2, Sarat Bose Road
Kolkata - 700020

Franchise : SGD 369

APOLLO CLINIC SARAT BOSE ROAD

Matru Chhaya Building

44/2 Sarat Bose Road, Kolkata-700 020

E-mail : saratboseroad@apolloclinic.com

Website : apollostrd.com

TO BOOK AN APPOINTMENT

Dr. Jyoti Kanti Das

MD (Path)

9038623111

9038623111

Reg No: WBMC-58338

7439475740(W)

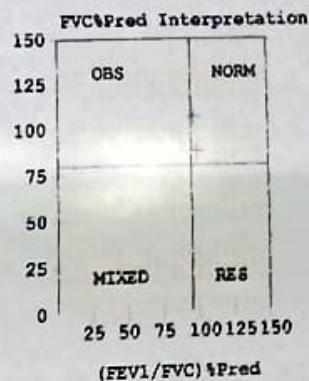
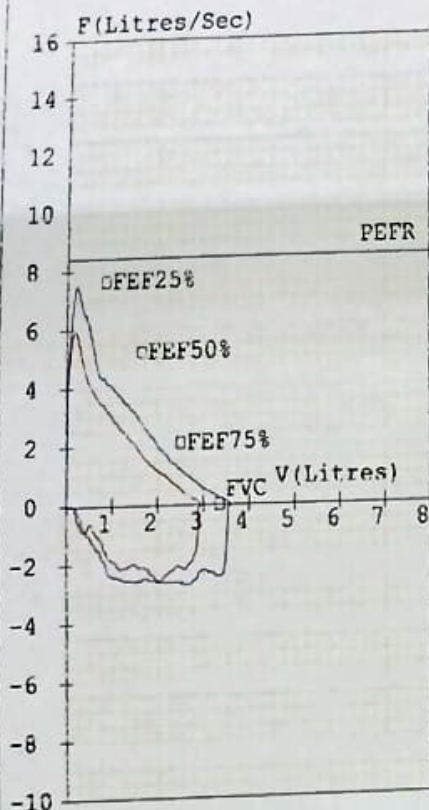
APOLLO CLINIC SARAT BOSE ROAD

44/2, SARAT BOSE ROAD KOLKATA-700020

Patient: SWAPAN BANERJEE
Refd. By: SELF
Pred. Eqns: RECORDERS
Date : 28-Oct-2025 11:50 AM

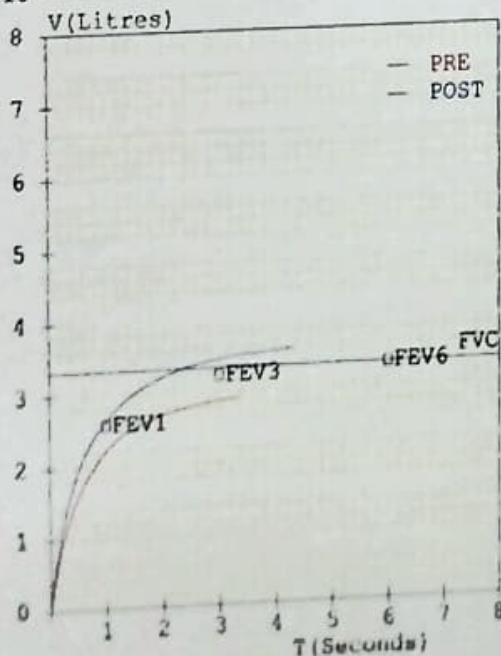
Age : 56 Yrs
Height : 172 Cms
Weight : 76 Kgs
ID : 20230

Gender : Male
Smoker : Yes
Eth. Corr: 100
Temp :



FVC Results

Parameter	Pred	M.Pre	%Pred	M.Post	%Pred	%Imp
FVC (L)	03.33	02.93	088	03.57	107	+22
FEV1 (L)	02.57	02.23	087	02.66	104	+19
FEV1/FVC (%)	77.18	76.11	099	74.51	097	-02
FEF25-75 (L/s)	03.26	01.80	055	02.04	063	+13
PEFR (L/s)	08.43	05.88	070	07.40	088	+26
FIVC (L)	-----	02.77	---	03.37	---	+22
FEV.5 (L)	-----	01.61	---	01.95	---	+21
FEV3 (L)	03.23	02.89	089	03.45	107	+19
PIFR (L/s)	-----	02.58	---	02.70	---	+05
FEF75-85 (L/s)	-----	00.69	---	00.69	---	---
FEF.2-1.2 (L/s)	05.90	03.55	060	04.78	081	+35
FEF 25% (L/s)	07.63	03.49	046	04.13	054	+18
FEF 50% (L/s)	05.19	02.11	041	02.44	047	+16
FEF 75% (L/s)	02.09	00.93	044	00.95	045	+02
FEV.5/FVC (%)	-----	54.95	---	54.62	---	-01
FEV3/FVC (%)	97.00	98.63	102	96.64	100	-02
FET (Sec)	-----	03.68	---	04.33	---	---
ExptTime (Sec)	-----	00.03	---	00.03	---	---
Lung Age (Yrs)	056	063	112	054	096	-14
FEV6 (L)	03.33	-----	---	02.23	---	+08
FIF25% (L/s)	-----	02.07	---	02.57	---	+22
FIF50% (L/s)	-----	02.11	---	02.52	---	+34
FIF75% (L/s)	-----	01.88	---	---	---	---



DR. RANAJAY CHANDA

APOLLO CLINIC SARAT BOSE ROAD
KOLKATA-700020
TEL: 033-2555-1111
FAX: 033-2555-1112

Pre Medication Report Indicates

Early Small Airway Obstruction as FEF 25-75 %Pred or PEFr %Pred < 70
Spirometry within normal limits as (FEV1/FVC) %Pred > 95 and FVC %Pred > 80

Post Medication Report Indicates

Early Small Airway Obstruction as FEF 25-75 %Pred or PEFr %Pred < 70
Spirometry within normal limits as (FEV1/FVC) %Pred > 95 and FVC %Pred > 80

DR. RANAJAY CHANDA

Patient Name	: Mr. SWAPAN BANERJEE	Advised/Pres Doctor	: --
Age/ Gender	: 56 Yrs / Male	Department	: Cardiology
UHID	: FSB.R.0000020230	Qualification	: --
OP Visit No.	: FSBROPV31858	Referred By	: Self
Printed On	: Oct 28, 2025, 2:04 PM	Registration No.	: --
Bill No	: FSB.R-OCR-25909	Employer Id	: 123

DEPARTMENT OF CARDIOLOGY

ELECTROCARDIOGRAM /ECG

Rate : 66 bpm
Rhythm : Sinus Arrhythmia
P-wave : Normal
Axis : Left Axis Deviation
PR-interval : 142 ms
QRS Interval : 90 ms
QT/QTc : 384/404 ms
ST-segment : No specific ST segment changes
QRS complex: Normal
T-wave : No specific 'T' wave changes
Q-wave : Insignificant
Ectopic : Nil

IMPRESSION :

- > Sinus Arrhythmia.
- > Left Axis Deviation.
- > No specific ST-T changes.

Note : Please clinically correlate.

---End Of The Report---



Dr. ANUP SHYAMAL
MBBS,MD,CONSULTANT CARDIOLOGIST
57185(WBMC)

Patient Name : Mr. SWAPAN BANERJEE**Advised/Pres Doctor** : --**Age/ Gender** : 56 Yrs / Male**Department** : Cardiology**UHID** : FSB.R.00000020230**Qualification** : --**OP Visit No.** : FSBROPV31858**Referred By** : Self**Printed On** : Oct 28, 2025, 2:04 PM**Registration No.** : --**Bill No** : FSB.R-OCR-25909**Employer Id** : 123

DEPARTMENT OF CARDIOLOGY

Cardiology



EMail:

255871 / SWAPAN BANERJEE / 56 Yrs / M / 172 Cms / 76 Kg

Date: 28 - 10 - 2025

Refd By: SELF Examined By:

NonCardiacPain Angina /Non-Hypercholesterolemia/Non-Diabetic/Negative Estrogen/Non-Athlete

Stage	Time	Duration	Speed(mph)	Elevation	METs	Rate	% THR	BP	RPP	PVC	Comments
Supine	00:16	0:16	00.0	00.0	01.0	064	39 %	139/87	088	00	
Standing	00:19	0:03	00.0	00.0	01.0	064	39 %	139/87	088	00	
HV	00:23	0:04	00.0	00.0	01.0	069	42 %	139/87	095	00	
Warm Up	00:27	0:04	01.0	00.0	01.0	078	48 %	139/87	108	00	
ExStart	00:50	0:23	01.0	00.0	01.0	089	54 %	139/87	123	00	
BRUCE Stage 1	03:50	3:00	01.7	10.0	04.7	120	73 %	150/87	180	00	
BRUCE Stage 2	06:50	3:00	02.5	12.0	07.1	137	84 %	160/90	219	00	
PeakEx	06:58	0:08	03.4	14.0	07.3	139	85 %	170/90	236	00	
Recovery	07:28	0:30	01.1	00.0	04.2	135	82 %	170/90	229	00	
Recovery	07:58	1:00	01.1	00.0	01.2	127	77 %	170/90	215	00	
Recovery	08:58	2:00	00.0	00.0	01.0	095	58 %	170/90	161	00	
Recovery	09:58	3:00	00.0	00.0	01.0	093	57 %	160/90	148	00	
Recovery	10:58	4:00	00.0	00.0	01.0	087	53 %	160/90	139	00	
Recovery	11:58	5:00	00.0	00.0	01.0	086	52 %	160/90	137	00	
Recovery	12:58	6:00	00.0	00.0	01.0	094	57 %	160/90	150	00	
Recovery	13:35	6:37	00.0	00.0	01.0	089	54 %	140/90	124	00	

FINDINGS :

Exercise Time : 06:08
 Initial HR (ExStrt) : 89 bpm 54% of Target 164
 Initial BP (ExStrt) : 139/87 (mm/Hg)
 Max WorkLoad Attained : 7.3 Fair response to induced stress
 Max ST Dep Lead & Avg ST Value : V6 & -1.0 mm in Stage 2
 Duke Treadmill Score : 06.2
 Test End Reasons : Test Complete, Heart Rate Achieved

Max HR Attained 139 bpm 85% of Target 164

Max BP Attained 170/90 (mm/Hg)



REPORT :

Swapan Banerjee

Dr. Anup Shyamal

Doctor : DR. ANUP SHYAMAL MBBS, MD

ID: 6
SWAPAN BANERJEE
Male 56Years
Req. No. :

Swapan Banerjee

28-10-2025 10:09:51 AM
HR : 66 bpm
P : 93 ms
PR : 142 ms
QRS : 90 ms
QT/QTcBz : 384/404 ms
P/QRS/T : 63/-66/58 °
RV5/SV1 : 1.113/0.000 mV

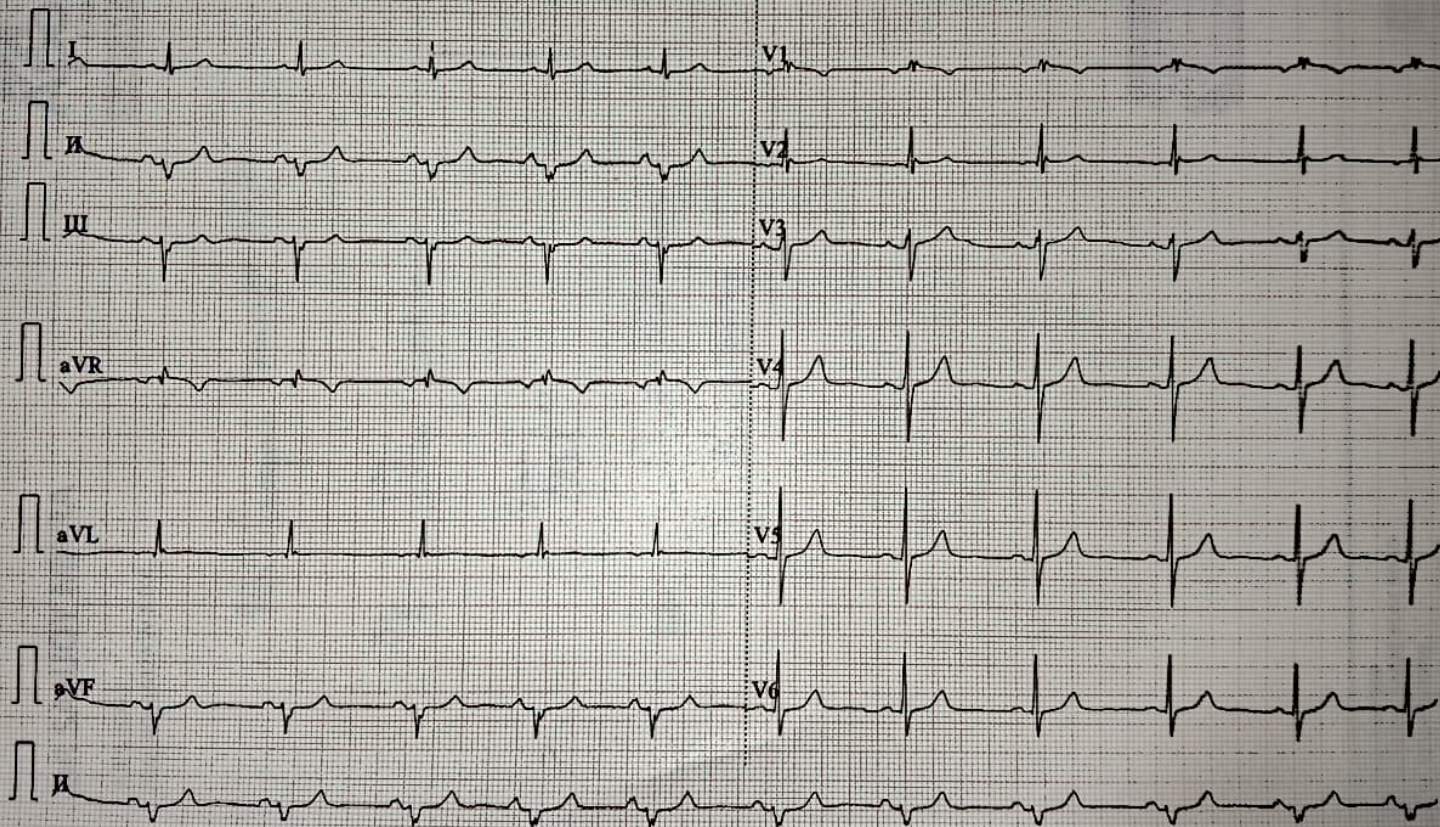
Diagnosis Information:

✓ Sinus Arrhythmia
Left Axis Deviation

Aditya Sharma

Ref-Phys. : SELF

Report Confirmed by:



DR SANJOY MITRA

B.D.S. (CAL)

REG. NO.-1632A W.B.D.C.

Mr. Swapan Banerjee 57/M.

7E — 17 distal caries.

Stann⁺⁺
seal⁺⁺

Adv: - i) — 17 R.C.T + crown
ii) ultrasonic scaling.

Shikha
28/10/25.

APOLLO CLINIC
44/2, Sarat Bose Road
Kolkata - 700020

Franchise : SGD 369

APOLLO CLINIC SARAT BOSE ROAD

Matru Chhaya Building

44/2 Sarat Bose Road, Kolkata 700020

email: saratboseroad@apolloclinic.com

Website : apollobrd.com

TO BOOK AN APPOINTMENT

 **033 3544 6062**

 **74394 75740**

POWER PRESCRIPTION

NAME : Swapan Banerjee

GENDER : M / F [✓] DATE : 27.10.25

AGE : 57y.

UHID :

RIGHT EYE

	SPH	CYL	AXIS	VISION
DISTANCE	+1.50	-0.50	30°	6/6
NEAR	+2.50	-	-	N ₆

LEFT EYE

	SPH	CYL	AXIS	VISION
DISTANCE	+1.00	-	-	6/6
NEAR	+2.50	-	-	N ₆

INSTRUCTIONS :

aided
VA < 6/6

- continue own Glass.
- Patient is colour Blind.
- Binocular Balance checked.
- Douochrome done.

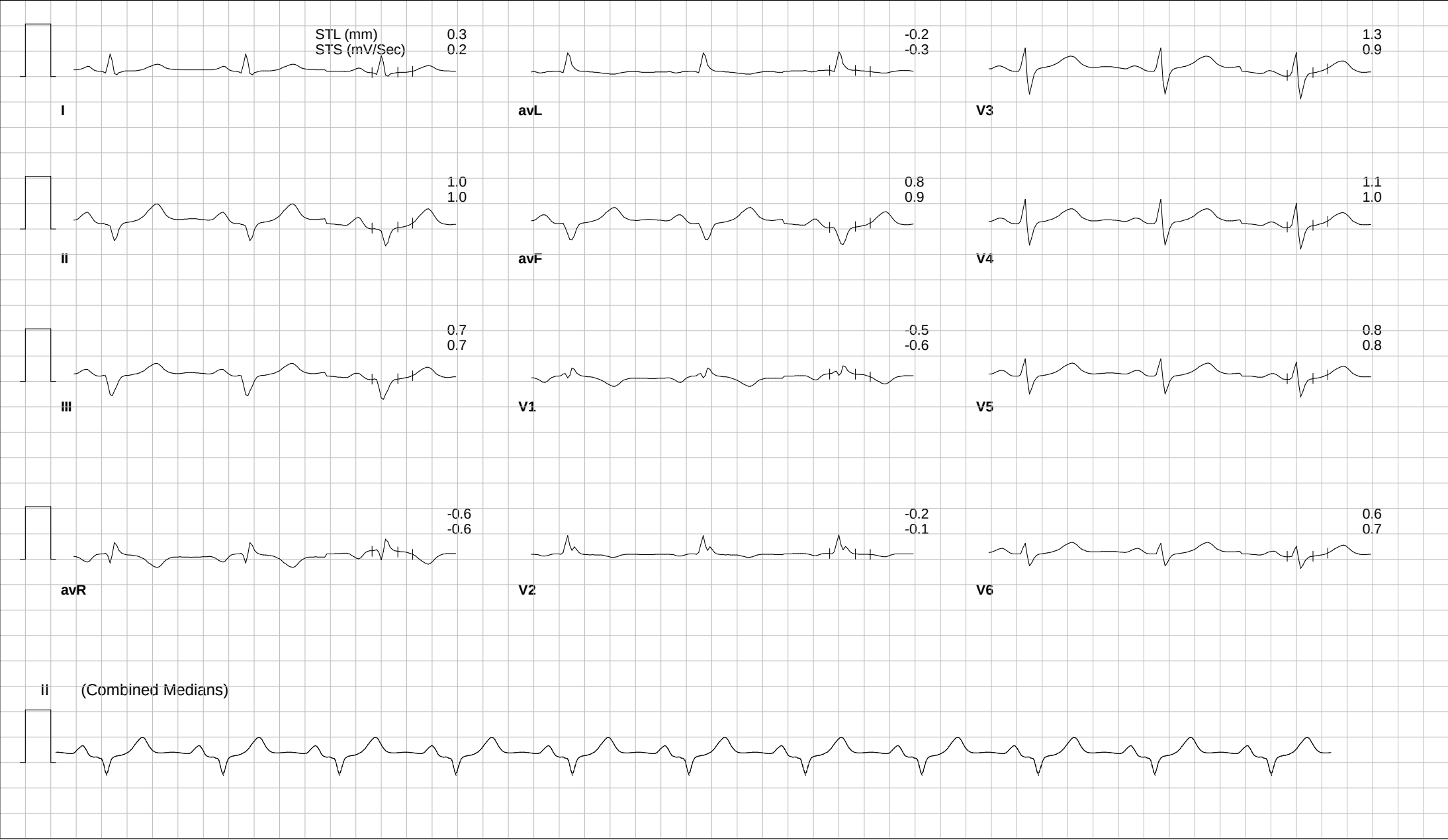
APOLLO CLINIC
44/2, Sarat Bose Road
Kolkata - 700020

R
27.10.25



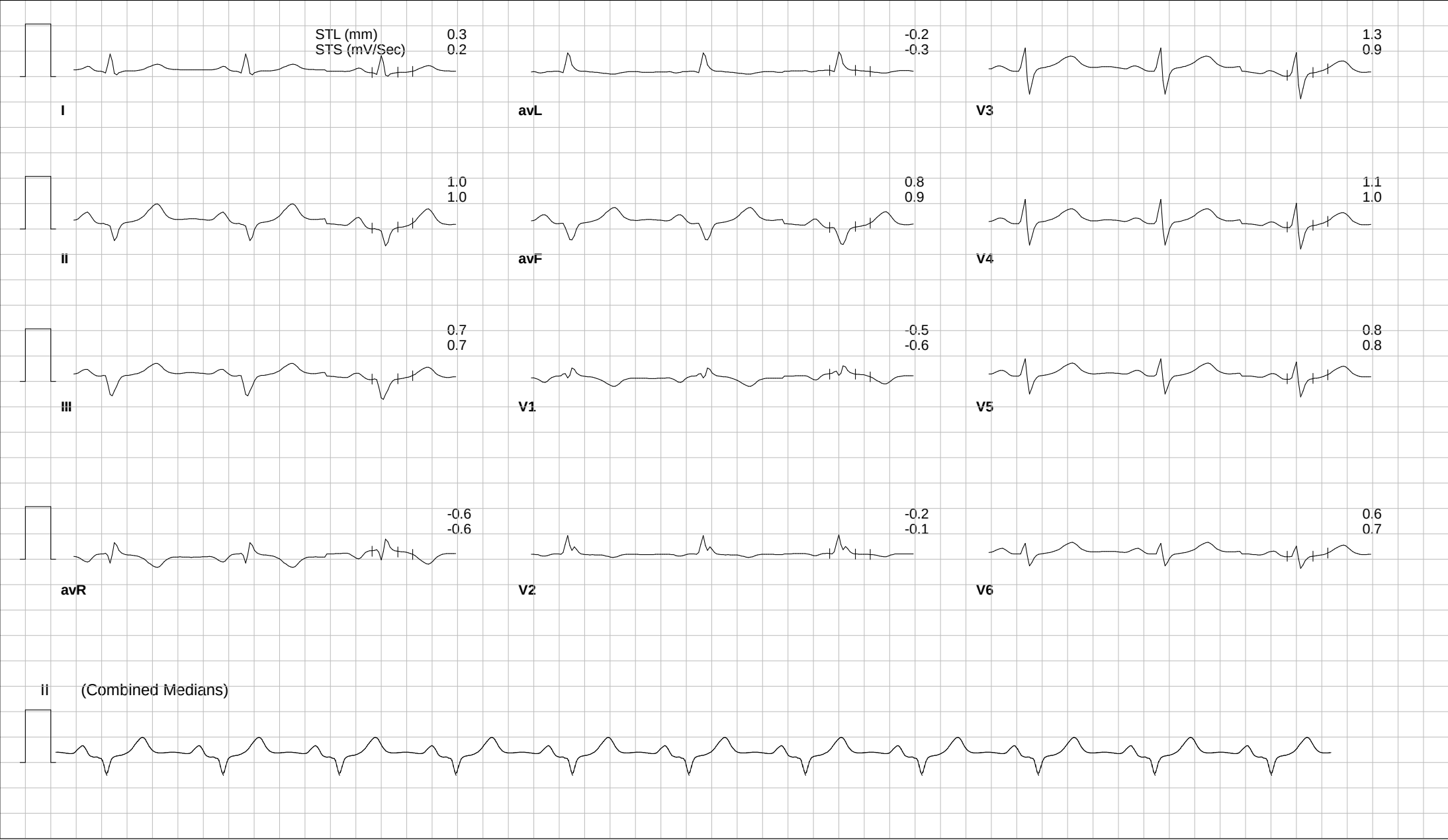


Date: 28 - 10 - 2025 METs : 1.0 HR : 64 Target HR : 39% of 164 BP : 139/87 Post J @80mSec ExTime: 00:00 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



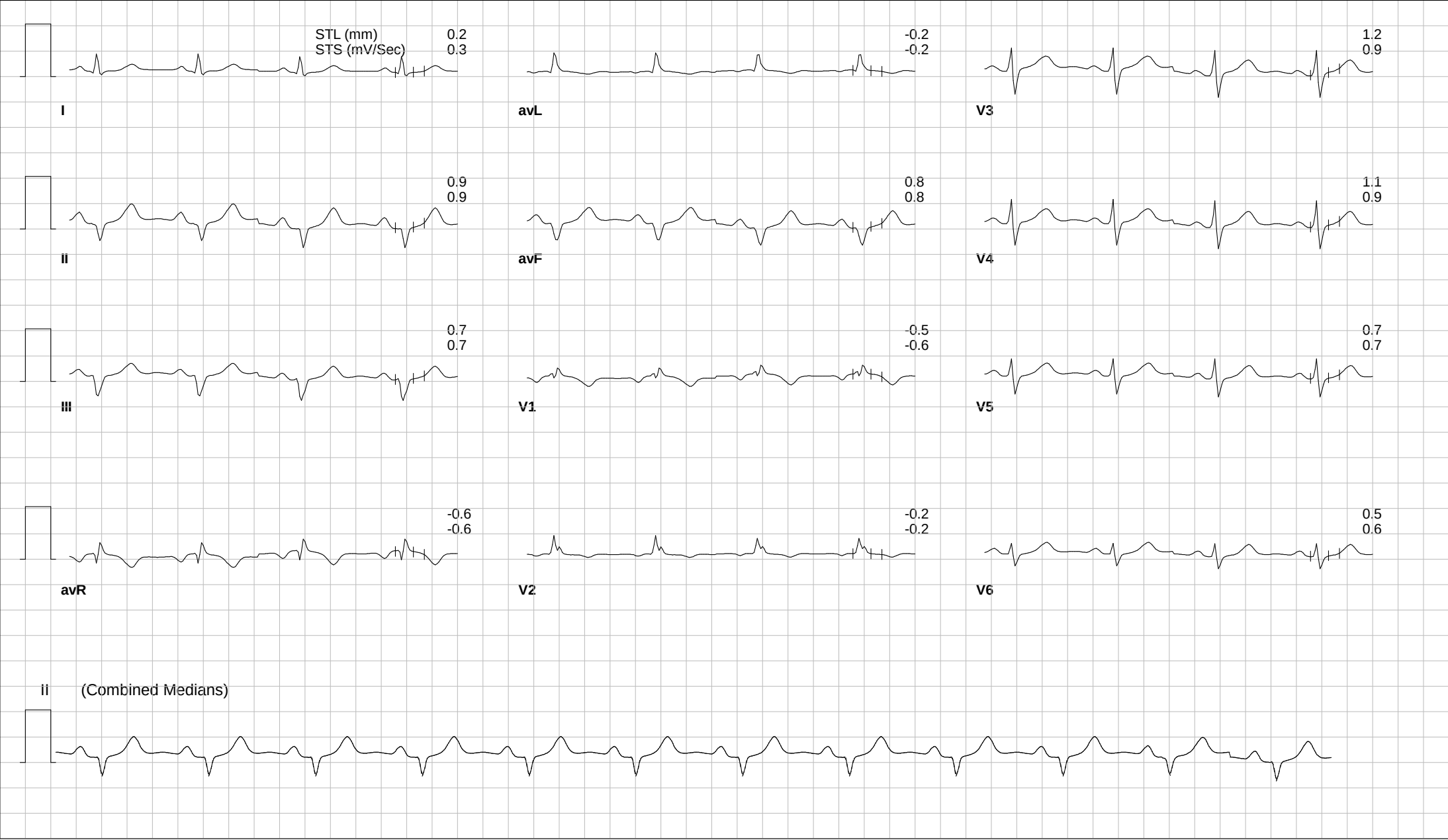


Date: 28 - 10 - 2025 METs : 1.0 HR : 64 Target HR : 39% of 164 BP : 139/87 Post J @80mSec ExTime: 00:00 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



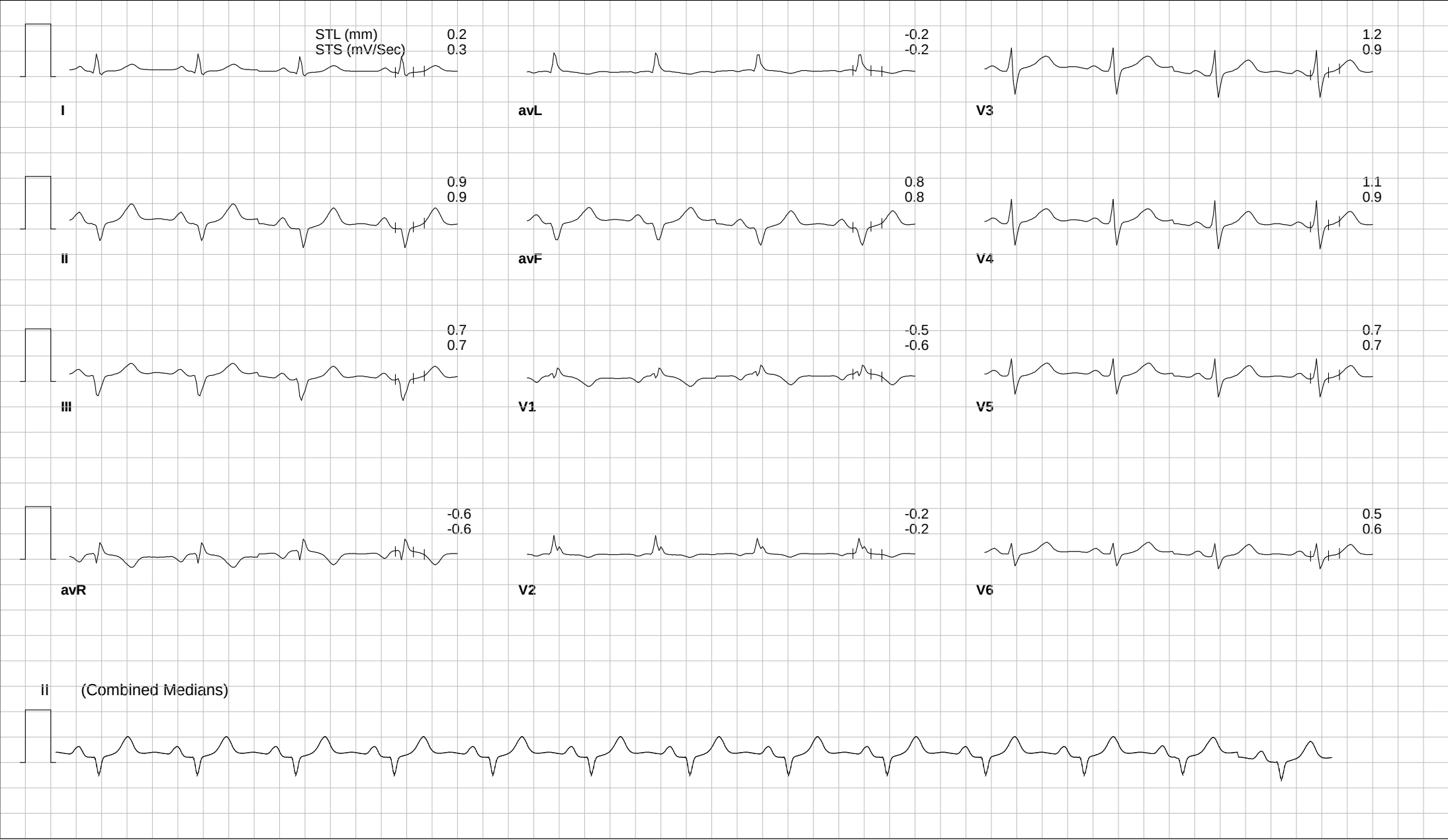


Date: 28 - 10 - 2025 METs : 1.0 HR : 69 Target HR : 42% of 164 BP : 139/87 Post J @80mSec ExTime: 00:00 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



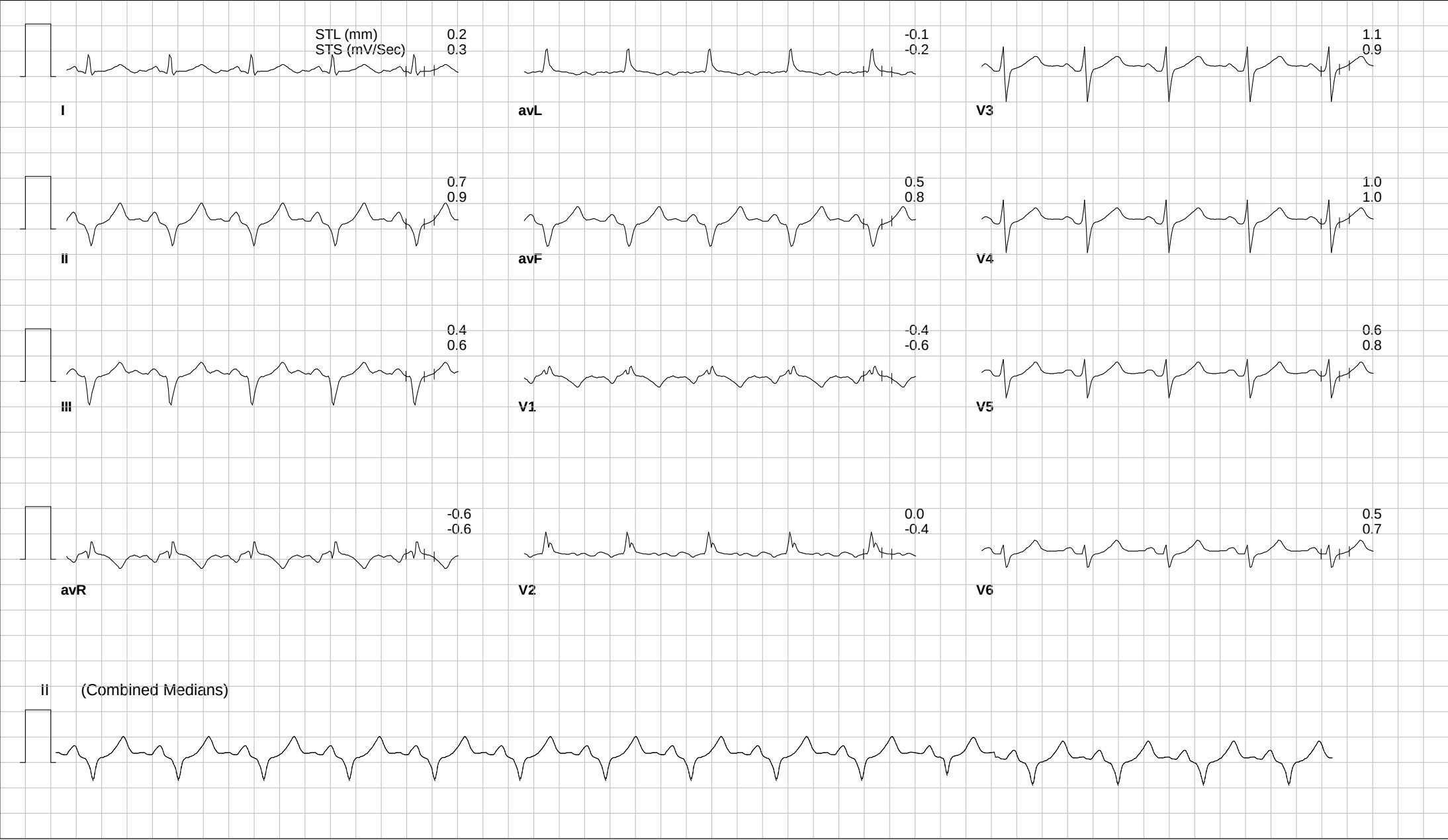


Date: 28 - 10 - 2025 METs : 1.0 HR : 78 Target HR : 48% of 164 BP : 139/87 Post J @80mSec ExTime: 00:00 Speed: 1.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



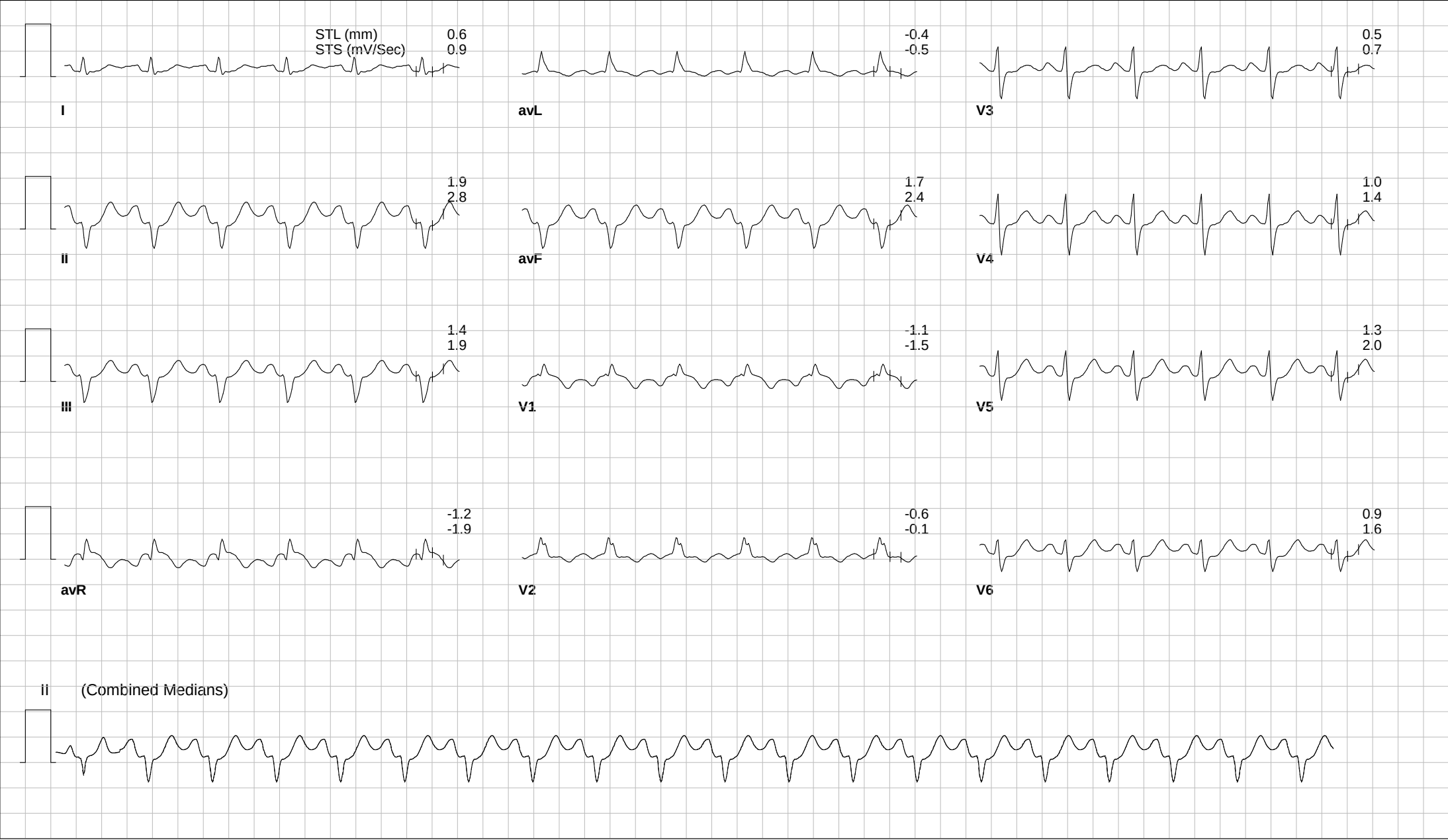


Date: 28 - 10 - 2025 METs : 1.0 HR : 89 Target HR : 54% of 164 BP : 139/87 Post J @80mSec ExTime: 00:00 Speed: 1.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



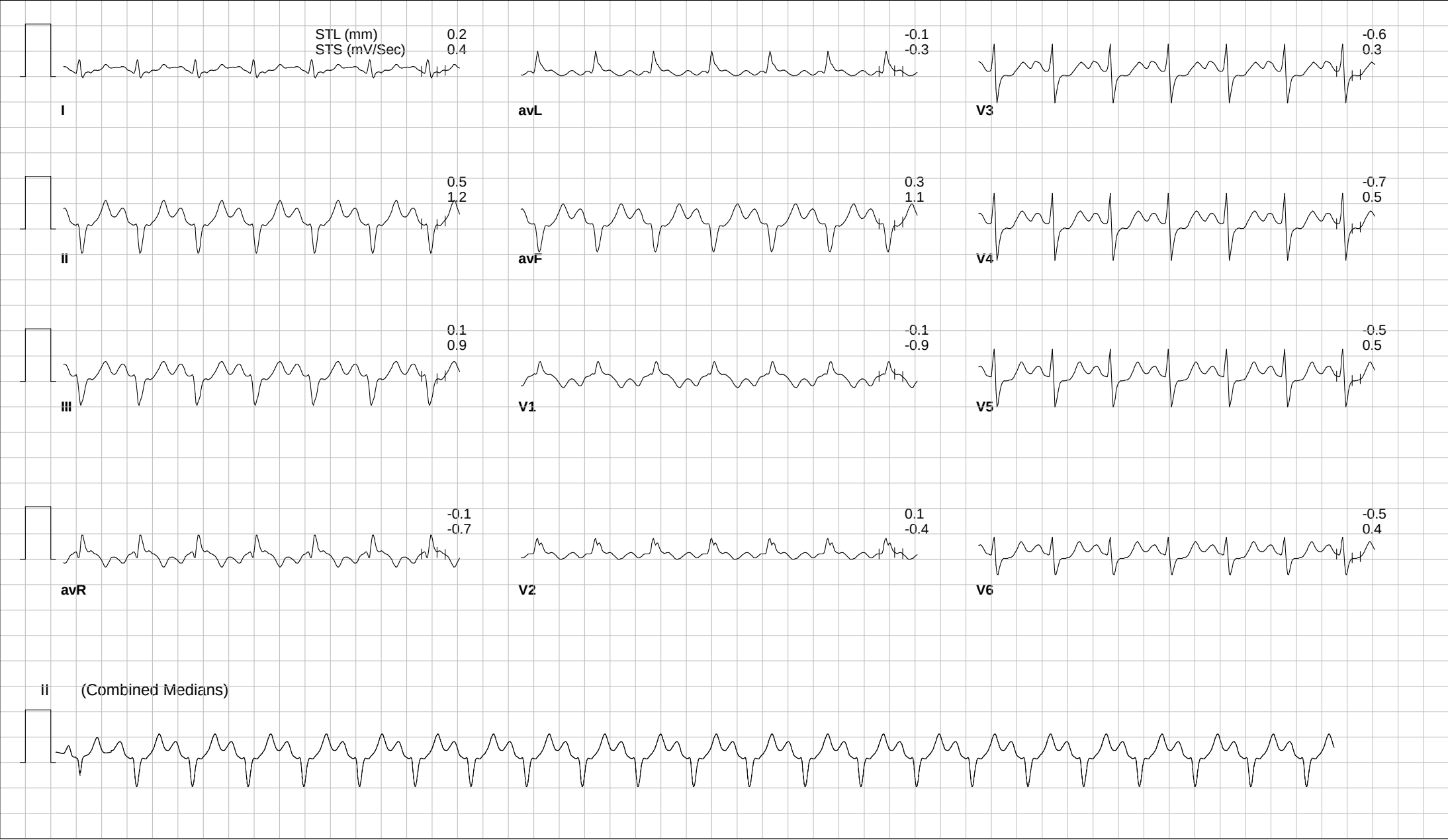


Date: 28 - 10 - 2025 METs : 4.7 HR : 120 Target HR : 73% of 164 BP : 150/87 Post J @80mSec ExTime: 03:00 Speed: 1.7 mph Grade : 10.00 % 25 mm/Sec. 1.0 Cm/mV



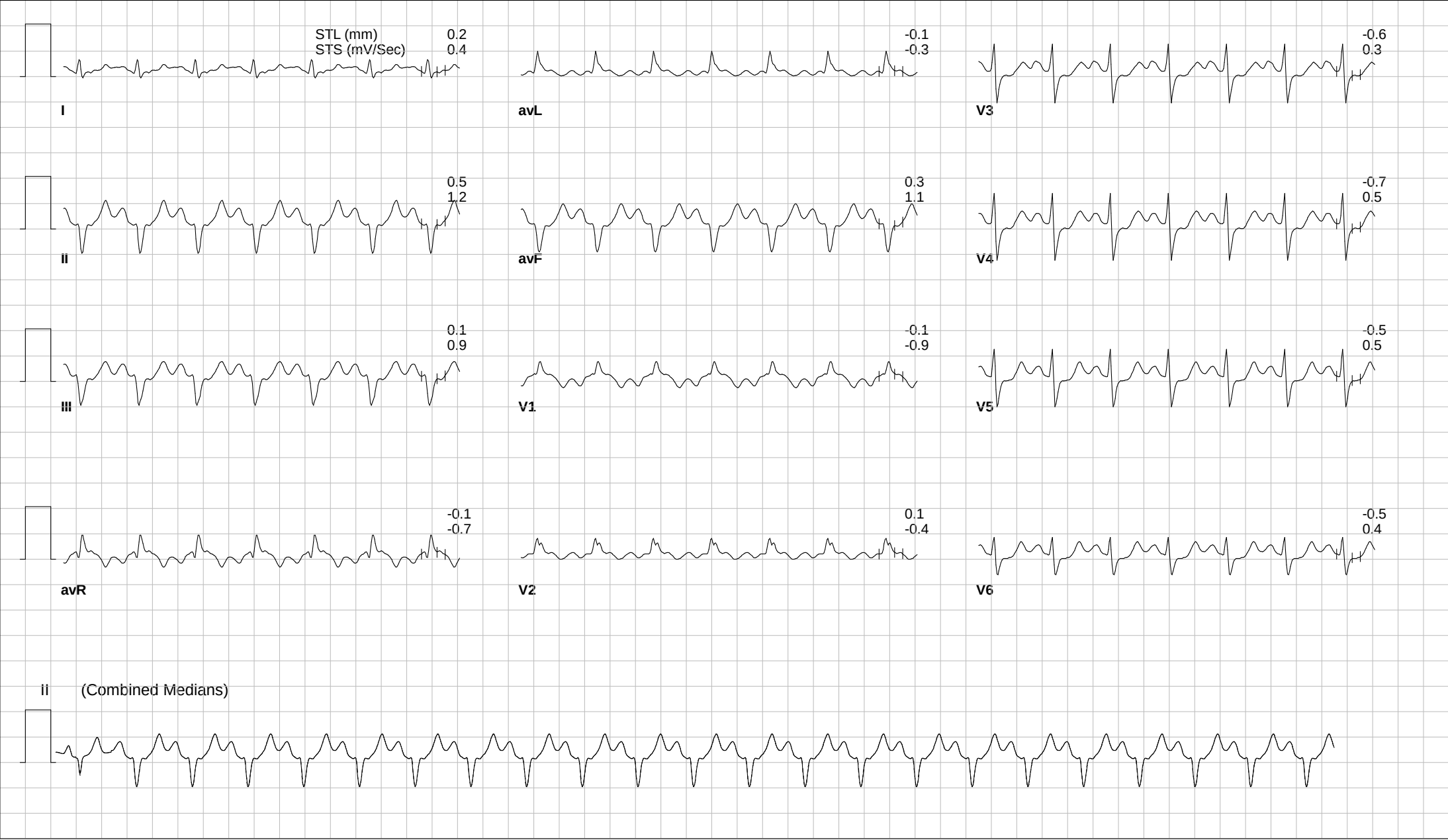


Date: 28 - 10 - 2025 METs : 7.1 HR : 137 Target HR : 84% of 164 BP : 160/90 Post J @60mSec ExTime: 06:00 Speed: 2.5 mph Grade : 12.00 % 25 mm/Sec. 1.0 Cm/mV



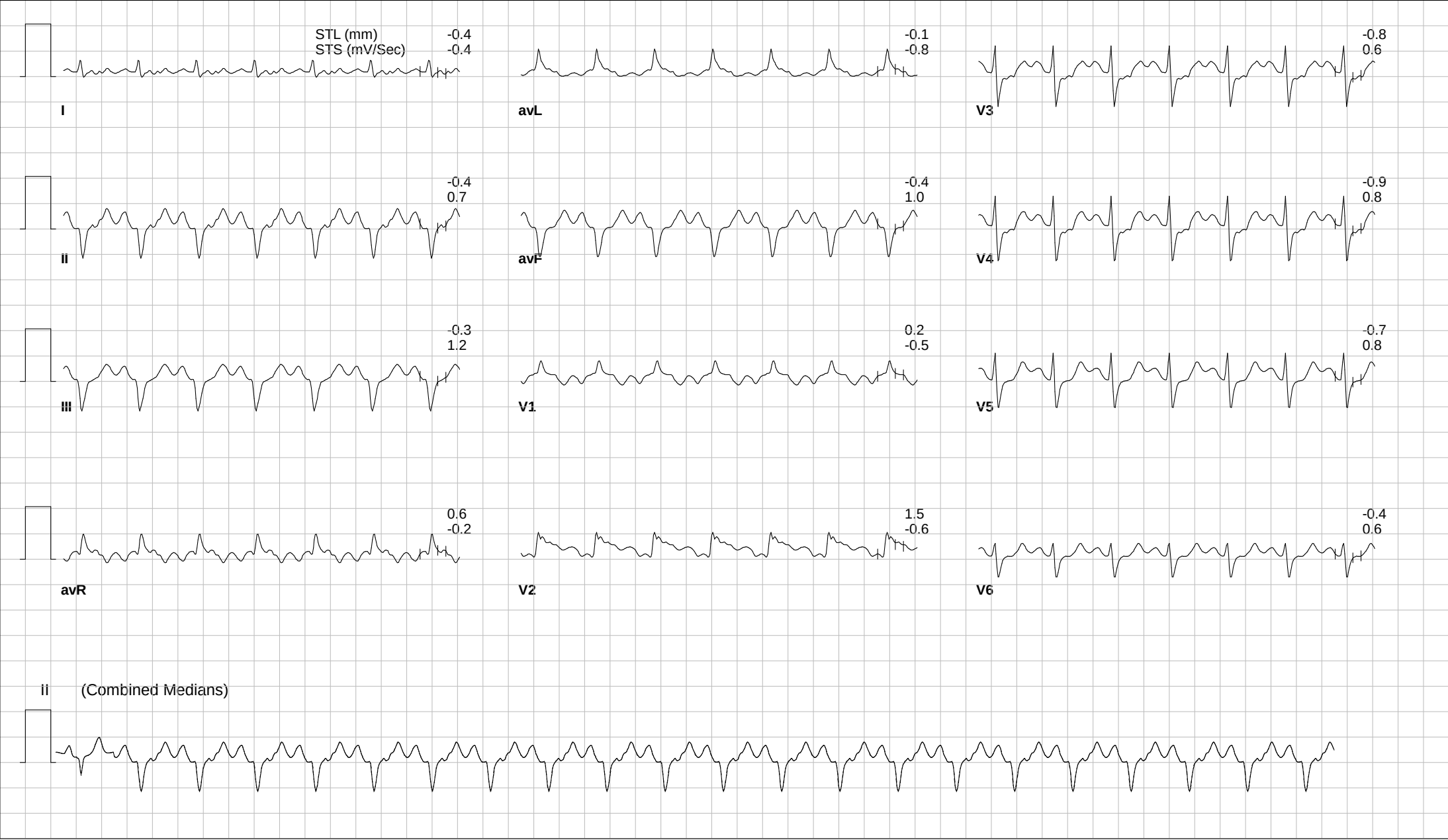


Date: 28 - 10 - 2025 METs : 7.3 HR : 139 Target HR : 85% of 164 BP : 170/90 Post J @80mSec ExTime: 06:08 Speed: 3.4 mph Grade : 14.00 % 25 mm/Sec. 1.0 Cm/mV



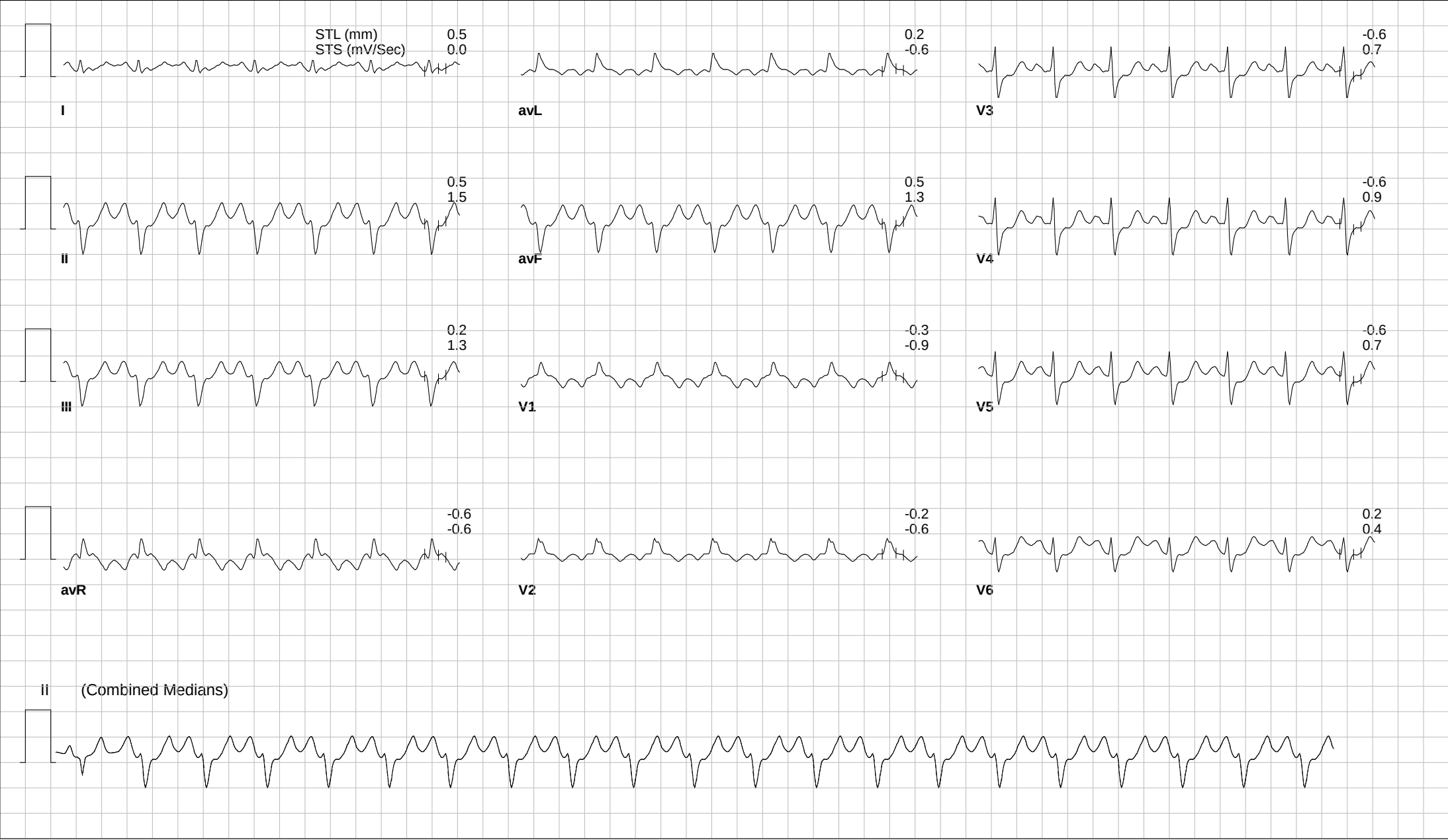


Date: 28 - 10 - 2025 METs : 4.2 HR : 135 Target HR : 82% of 164 BP : 170/90 Post J @60mSec ExTime: 06:08 Speed: 1.1 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



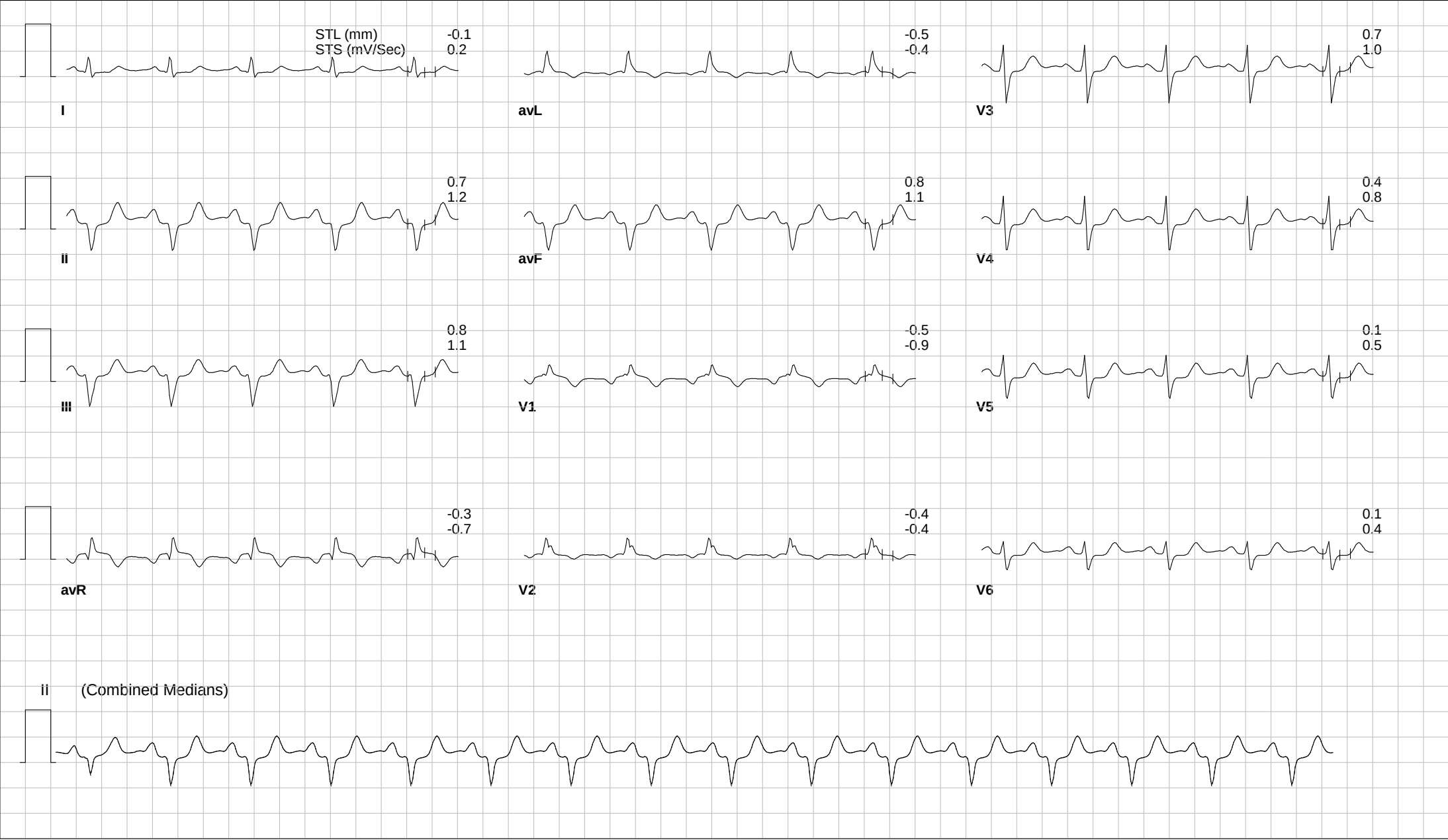


Date: 28 - 10 - 2025 METs : 1.2 HR : 127 Target HR : 77% of 164 BP : 170/90 Post J @60mSec ExTime: 06:08 Speed: 1.1 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



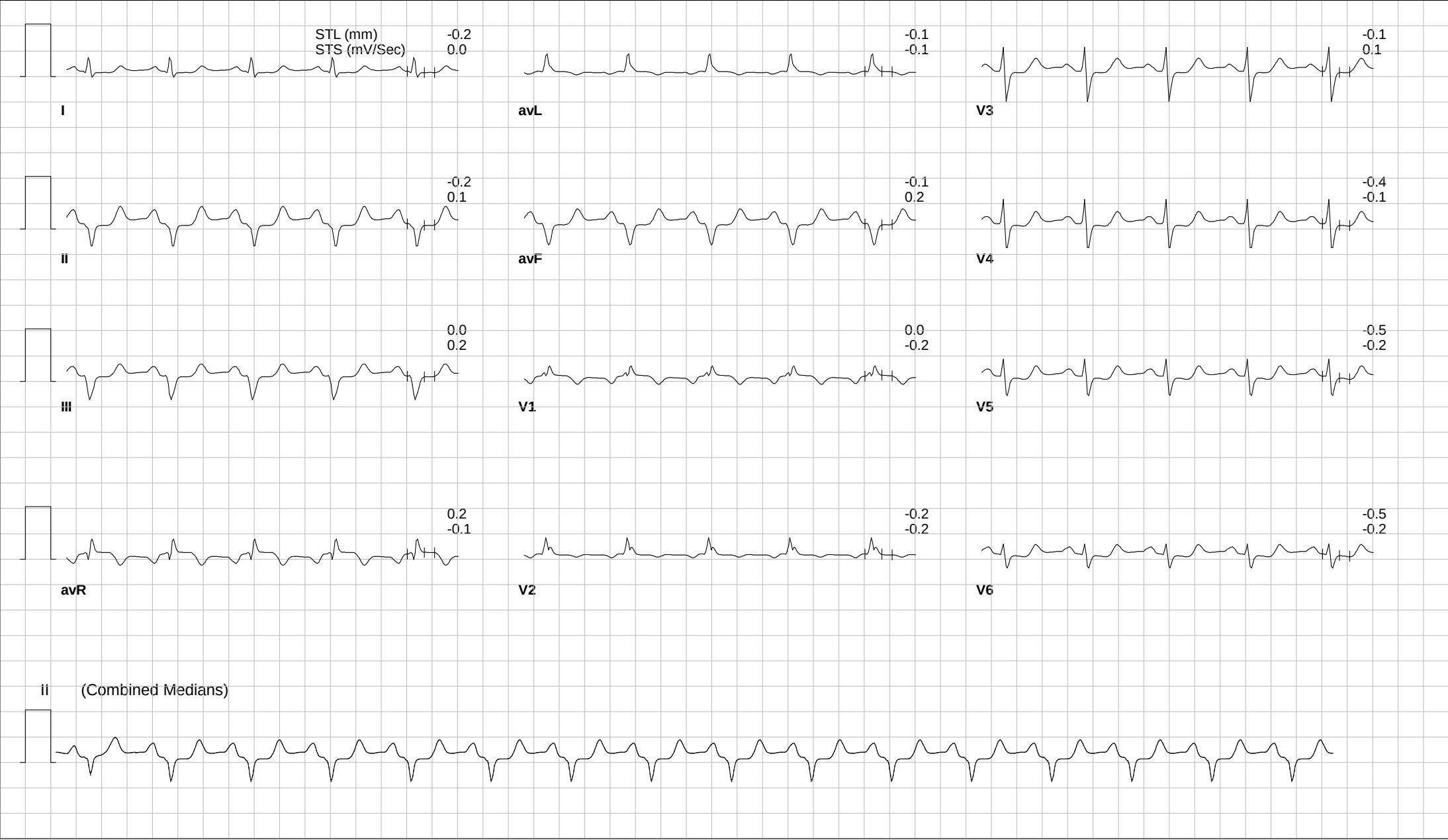


Date: 28 - 10 - 2025 METs : 1.0 HR : 95 Target HR : 58% of 164 BP : 170/90 Post J @80mSec ExTime: 06:08 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



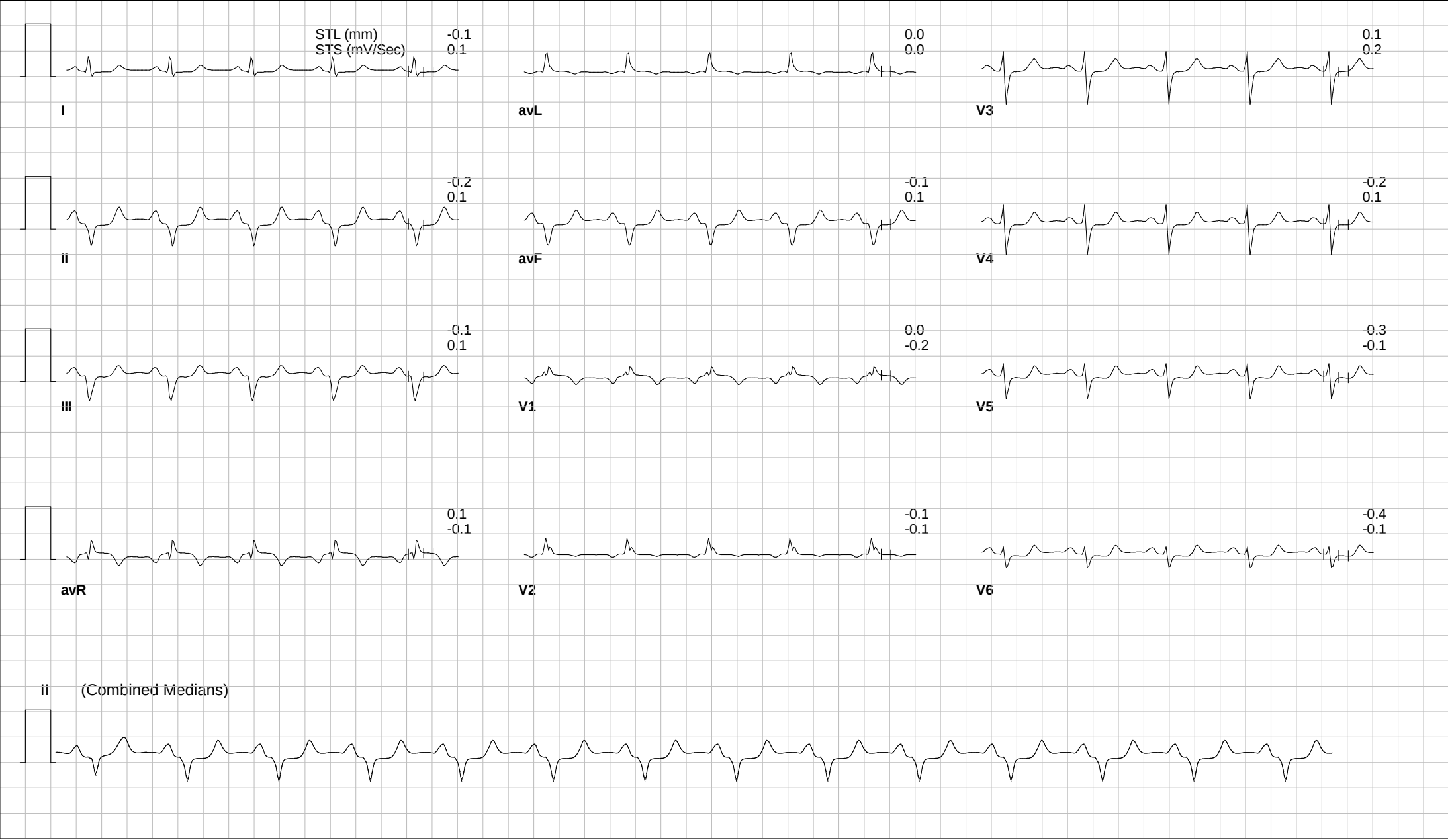


Date: 28 - 10 - 2025 METs : 1.0 HR : 93 Target HR : 57% of 164 BP : 160/90 Post J @80mSec ExTime: 06:08 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



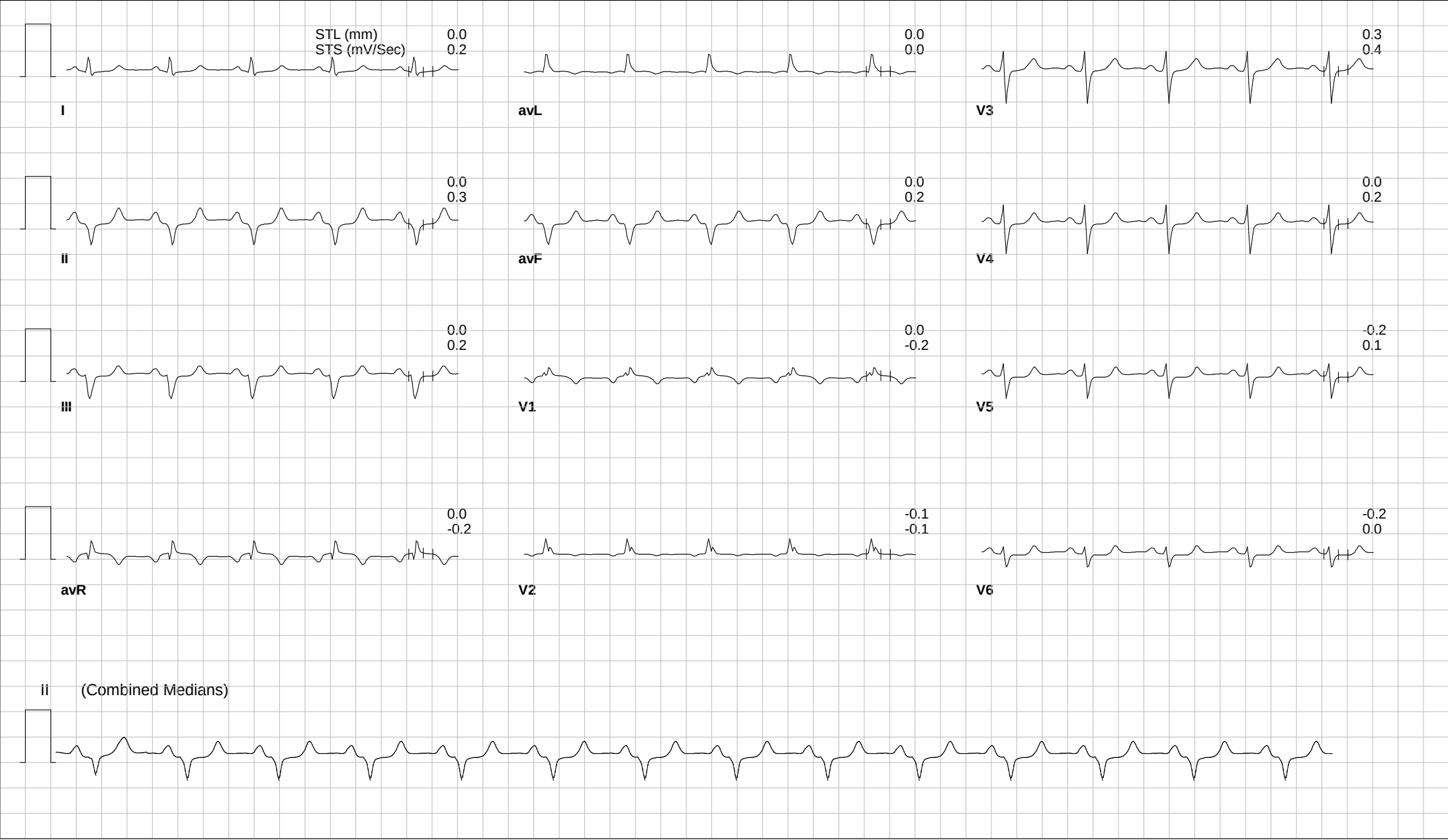


Date: 28 - 10 - 2025 METs : 1.0 HR : 87 Target HR : 53% of 164 BP : 160/90 Post J @80mSec ExTime: 06:08 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



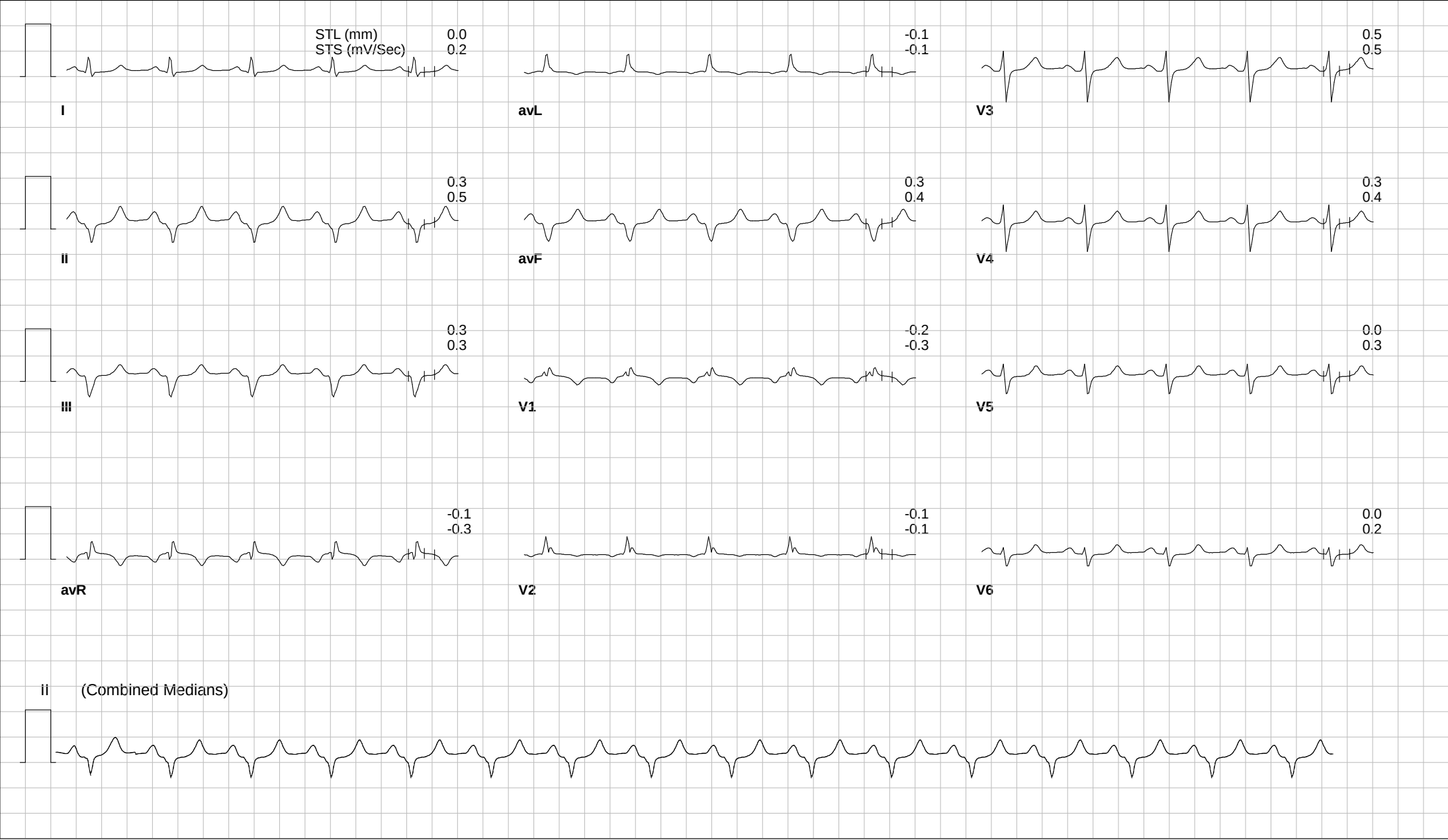


Date: 28 - 10 - 2025 METs : 1.0 HR : 86 Target HR : 52% of 164 BP : 160/90 Post J @80mSec ExTime: 06:08 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



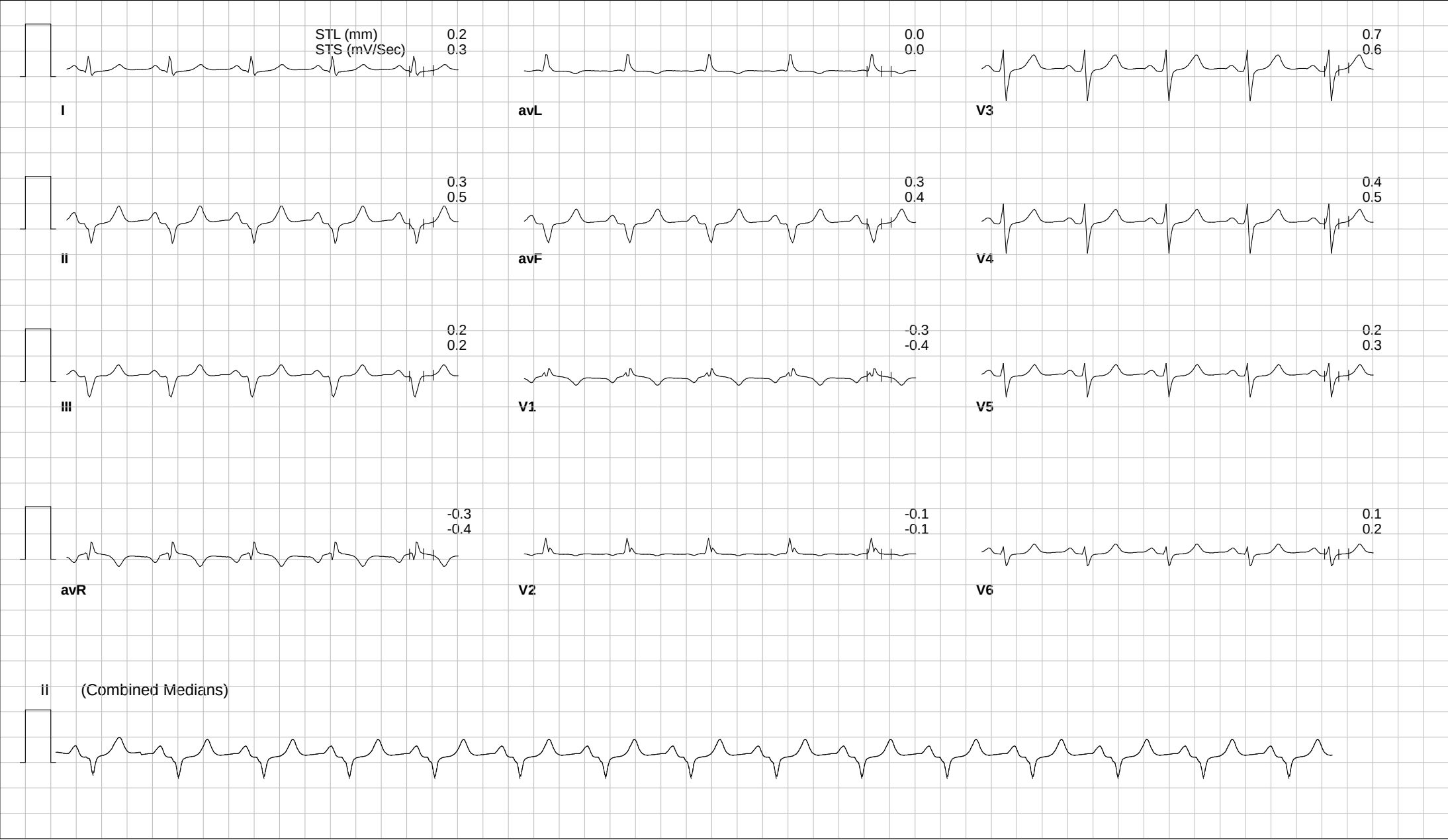


Date: 28 - 10 - 2025 METs : 1.0 HR : 94 Target HR : 57% of 164 BP : 160/90 Post J @80mSec ExTime: 06:08 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV





Date: 28 - 10 - 2025 METs : 1.0 HR : 89 Target HR : 54% of 164 BP : 140/90 Post J @80mSec ExTime: 06:08 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV





E-Mail:

255871 / SWAPAN BANERJEE / 56 Yrs / M / 172 Cms / 76 Kg

Date: 28 - 10 - 2025

Refd By : SELF

Examined By:

NonCardiacPain Angina /Non-Hypercholestromia/Non-Diabetic/Negative Estrogen/Non-Athlete

Stage	Time	Duration	Speed(mph)	Elevation	METs	Rate	% THR	BP	RPP	PVC	Comments
Supine	00:16	0:16	00.0	00.0	01.0	064	39 %	139/87	088	00	
Standing	00:19	0:03	00.0	00.0	01.0	064	39 %	139/87	088	00	
HV	00:23	0:04	00.0	00.0	01.0	069	42 %	139/87	095	00	
Warm Up	00:27	0:04	01.0	00.0	01.0	078	48 %	139/87	108	00	
ExStart	00:50	0:23	01.0	00.0	01.0	089	54 %	139/87	123	00	
BRUCE Stage 1	03:50	3:00	01.7	10.0	04.7	120	73 %	150/87	180	00	
BRUCE Stage 2	06:50	3:00	02.5	12.0	07.1	137	84 %	160/90	219	00	
PeakEx	06:58	0:08	03.4	14.0	07.3	139	85 %	170/90	236	00	
Recovery	07:28	0:30	01.1	00.0	04.2	135	82 %	170/90	229	00	
Recovery	07:58	1:00	01.1	00.0	01.2	127	77 %	170/90	215	00	
Recovery	08:58	2:00	00.0	00.0	01.0	095	58 %	170/90	161	00	
Recovery	09:58	3:00	00.0	00.0	01.0	093	57 %	160/90	148	00	
Recovery	10:58	4:00	00.0	00.0	01.0	087	53 %	160/90	139	00	
Recovery	11:58	5:00	00.0	00.0	01.0	086	52 %	160/90	137	00	
Recovery	12:58	6:00	00.0	00.0	01.0	094	57 %	160/90	150	00	
Recovery	13:35	6:37	00.0	00.0	01.0	089	54 %	140/90	124	00	

FINDINGS :

Exercise Time	: 06:08	
Initial HR (ExStrt)	: 89 bpm 54% of Target 164	Max HR Attained 139 bpm 85% of Target 164
Initial BP (ExStrt)	: 139/87 (mm/Hg)	Max BP Attained 170/90 (mm/Hg)
Max WorkLoad Attained	: 7.3 Fair response to induced stress	
Max ST Dep Lead & Avg ST Value	: V6 & -1.0 mm in Stage 2	
Duke Treadmill Score	: 06.2	
Test End Reasons	: Test Complete, Heart Rate Achieved	

REPORT :

Doctor : DR.ANUP SHYAMAL MBBS,MD

Patient Name	: Mr. SWAPAN BANERJEE	Advised/Pres Doctor	: --
Age/ Gender	: 56 Yrs / Male	Department	: Cardiology
UHID	: FSB.R.0000020230	Qualification	: --
OP Visit No.	: FSBROPV31858	Referred By	: Self
Printed On	: Oct 28, 2025, 1:03 PM	Registration No.	: --
Bill No	: FSB.R-OCR-25909	Employer Id	: 123

DEPARTMENT OF CARDIOLOGY

TREADMILL TEST / TMT

Resting ECG showing normal sinus rhythm.

During TMT patient showed normal chronotropic & hemodynamic response.

Test End Reason :Target heart rate achieved.

Exercise tolerance was fair.(METs = 7.3)

Patient was asymptomatic during exercise.

No Significant ST-T changes noted during test.

No Pathological arrhythmia was detected during the procedure.

IMPRESSION:

TMT is negative for provokable myocardial ischemia.

Note : Please clinically correlate.

---End Of The Report---



Dr. ANUP SHYAMAL
MBBS,MD,CONSULTANT CARDIOLOGIST
57185(WBMC)
Cardiology

Patient Name	: Mr. SWAPAN BANERJEE	Advised/Pres Doctor	: --
Age/ Gender	: 56 Yrs / Male	Department	: Radiology
UHID	: FSBR.0000020230	Qualification	: --
OP Visit No.	: FSBROPV31858	Referred By	: Self
Printed On	: Oct 28, 2025, 12:57 PM	Registration No.	: --
Bill No	: FSBR-OCR-25909	Employer Id	: 123

DEPARTMENT OF RADIOLOGY

Ultrasound Whole Abdomen (Male)

LIVER : Liver is normal in size and fatty parenchymal echo pattern. No focal intra-hepatic lesion detected.

Intra-hepatic biliary radicals are not dilated. Portal vein is normal in calibre measure 0.8 cm . Common duct is not dilated measure 0.5 cm .

GALL BLADDER : Gall bladder is well distended . Wall is normal thickness. Lumen is clear.

PANCREAS : Pancreas is normal in size and echo pattern.

SPLEEN : Spleen is normal in size (11.6 cm) & echo pattern . Splenic vein is not dilated .

KIDNEYS : Both kidneys are normal in position, size and outline. Cortico-medullary differentiation of both kidneys is maintained. Central sinus echoes are compact. Right kidney measure- 9.5 cm . Left kidney measure - 8.8 cm .

Visualized parts of retro-peritoneum do not reveal any lymphadenopathy.
No significant free fluid is detected .

URINARY BLADDER : Urinary bladder is partially distended , in wall thickness with clear contents.

No significant intra or extraluminal mass seen.

Patient Name : Mr. SWAPAN BANERJEE**Advised/Pres Doctor** : --**Age/ Gender** : 56 Yrs / Male**Department** : Radiology**UHID** : FSB.R.0000020230**Qualification** : --**OP Visit No.** : FSBROPV31858**Referred By** : Self**Printed On** : Oct 28, 2025, 12:57 PM**Registration No.** : --**Bill No** : FSB.R-OCR-25909**Employer Id** : 123

DEPARTMENT OF RADIOLOGY

PROSTATE : Prostate is normal in size and echopattern . Prostate measures 2.5 x 3.1 x 3.1 cm ,VOL- 13.1 cc.

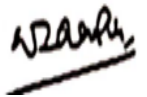
A Hernia seen in umbilical region measures 5.1 x 4.3 cm .

IMPRESSION : 1. Grade I fatty changes in liver .

2. Umbilical hernia .

Suggest : Further investigation and follow up if indicated.

---End Of The Report---



Dr. M NURUZZAMAN

MBBS, DMRD, MD

45591(WBMC)

Radiology

SWAPAN BANERJEE 56 YR / M, (MRN 202250)

PHILIPS



20/10/2025 09:54 AM

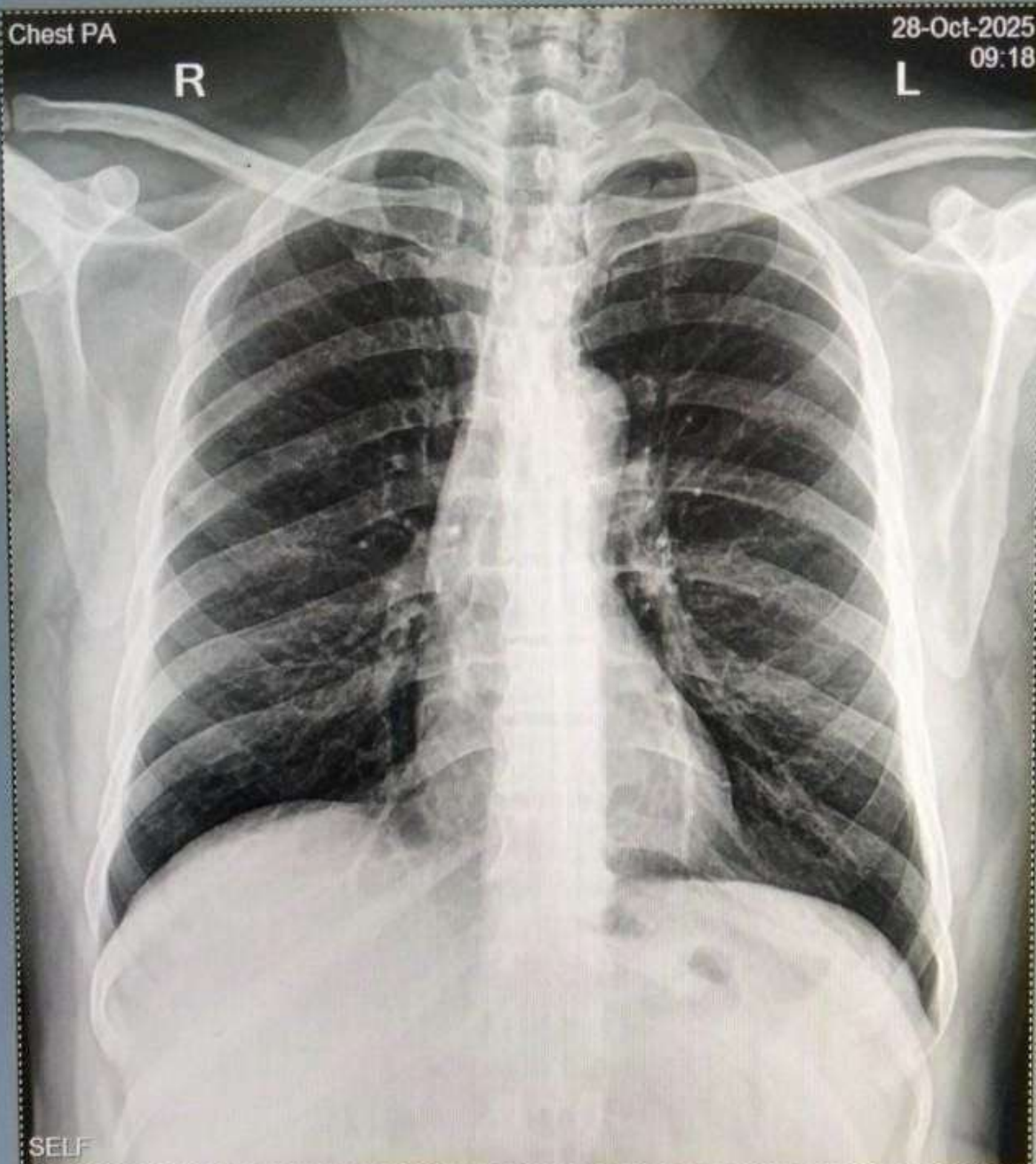


Chest PA

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SELF

00, SWAPAN BANERJEE, M, 56 years

APOLLO CLINIC (Licensee-SGD 369) ,44/2,SARAT BOSE ROAD

Patient Name : Mr. SWAPAN BANERJEE**Advised/Pres Doctor** : --**Age/ Gender** : 56 Yrs / Male**Department** : Radiology**UHID** : FSB.R.0000020230**Qualification** : --**OP Visit No.** : FSBROPV31858**Referred By** : Self**Printed On** : Oct 28, 2025, 11:17 AM**Registration No.** : --**Bill No** : FSB.R-OCR-25909**Employer Id** : 123

DEPARTMENT OF RADIOLOGY

X-RAY CHEST PA**FINDINGS:**

Lung parenchyma shows no focal lesion.

Bronchovascular lung markings are within normal limits.

Both of hila are normal in size, density and position.

Domes of diaphragm are smoothly outlined.

Bilateral costophrenic angles are clear.

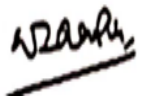
Cardiac size appears within normal limits.

Bony thorax reveals no definite abnormality.

IMPRESSION: Normal Study.

Note: Please clinically correlate.

---End Of The Report---



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